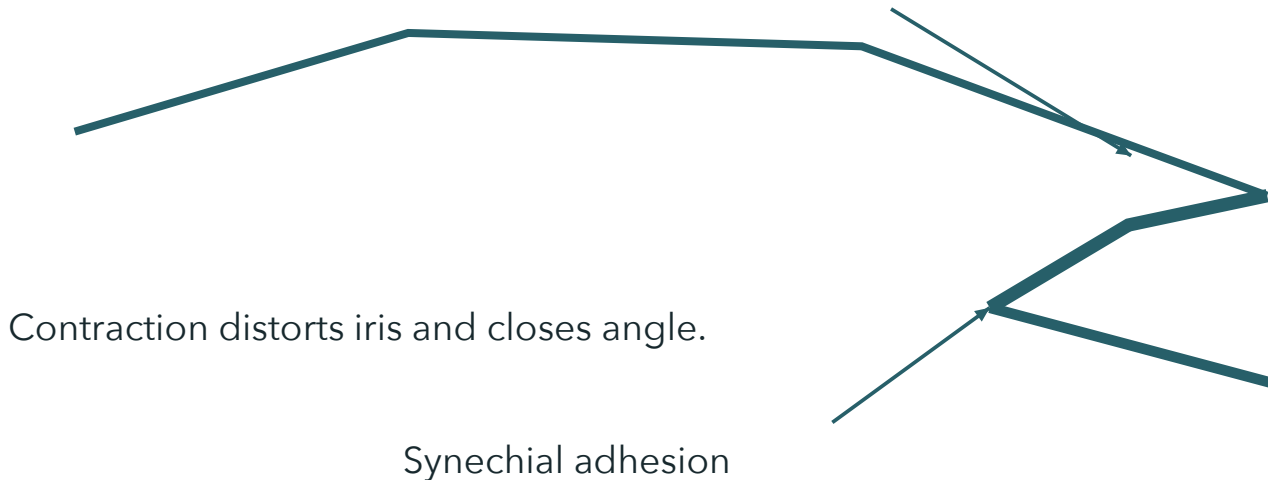


Secondary glaucoma beyond pupillary block

Part 2 | Patent iridotomy does not exclude angle closure

A membrane can contract across the angle without a pupil-block gradient.

Abnormal endothelial membrane



Original section: abnormal endothelial membrane extends towards the angle and pulls the iris.

ICE is usually unilateral and can combine abnormal endothelium, iris distortion and progressive peripheral anterior synechiae.

Gonioscopy and endothelial assessment connect the structural findings. Pressure treatment may become surgically complex as synechiae progress. An iridotomy treats a pupil-block component, not an endothelial membrane.

Blood in Schlemm canal can be a venous clue

Look for the downstream pressure source rather than treating an isolated trabecular label.

Finding	Interpretation
Corkscrew episcleral vessels	Consider elevated venous pressure
Blood reflux in Schlemm canal	Downstream pressure can reverse the gradient
Proptosis or vascular orbital signs	Investigate a fistula or other venous cause
Unilateral post-traumatic onset	History changes the investigation priority

Control IOP while investigating the cause with appropriate orbital or vascular assessment. Idiopathic elevated episcleral venous pressure is an exclusion diagnosis. A normal-looking pupil and open angle do not establish primary open-angle glaucoma when the venous examination is discordant.

Steroid response and inflammation can coexist

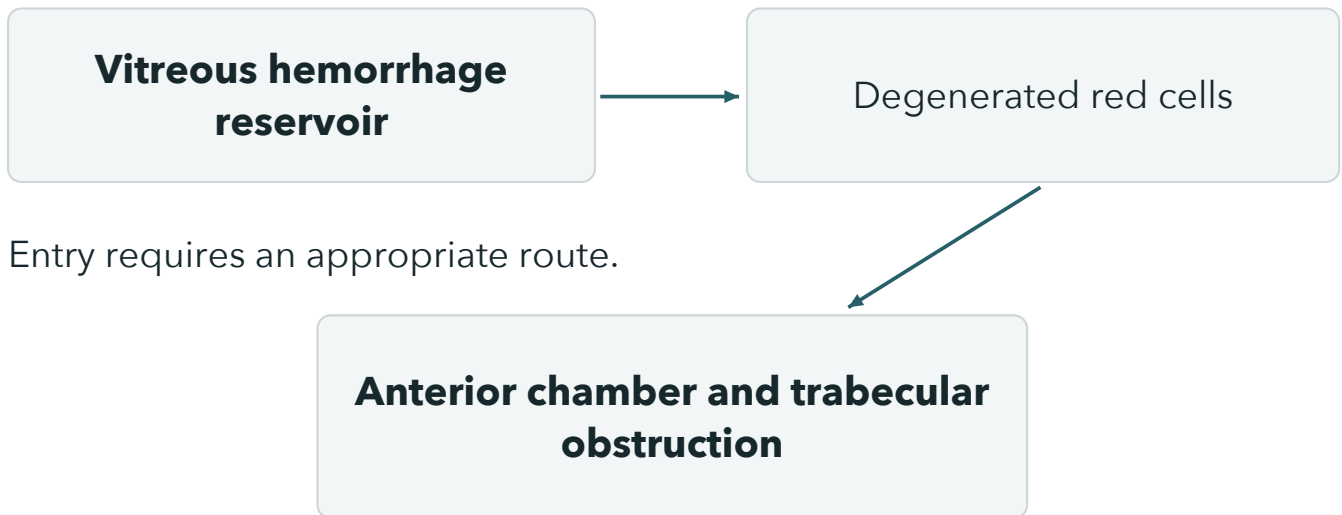
Reducing the harmful exposure must not abandon essential disease control.

Review	Reasoning
Every steroid route and potency	Ocular and nonocular exposures may contribute
Timing and previous response	Risk is not confined to one fixed interval
Optic nerve and fields	Ocular hypertension differs from glaucoma
Inflammatory requirement	Coordinate reduction, substitution or pressure treatment

A pressure rise during treatment is not proof that all inflammation has resolved. Use the least harmful adequate anti-inflammatory strategy with monitoring. Some eyes retain elevated IOP after withdrawal, particularly with prolonged exposure or damage; pressure follow-up cannot end automatically on the day the steroid stops.

Ghost cells can arrive from a posterior reservoir

Anterior obstruction can recur if the source remains.



Entry requires an appropriate route.

Original compartment map: altered red cells reach the angle from a vitreous hemorrhage reservoir.

Consider ghost-cell glaucoma after vitreous hemorrhage when compatible degenerated cells appear in the anterior chamber. Treat pressure while assessing the source and route. Repeated anterior washout may not prevent recurrence if the vitreous reservoir persists; selected refractory cases require posterior source treatment rather than an endless sequence of chamber washes.

Epithelial downgrowth is not LASIK ingrowth

A wound-associated intraocular membrane can obstruct and distort the angle.

Feature	Implication
Prior penetrating wound or operation	Possible epithelial entry route
Retrocorneal or iris membrane	Define extent and progression
Secondary glaucoma	Multiple outflow mechanisms may coexist
Progressive tissue involvement	Requires specialist diagnostic and treatment planning

Distinguish intraocular epithelial downgrowth from epithelial cells confined to a LASIK interface. The extent, cystic versus diffuse pattern and associated complications determine treatment. A generic anti-inflammatory response does not prove control of a proliferating epithelial membrane, and delay can make later intervention harder.